

Patient ID: WimberleyPeriod: December 7 to February 6, 2024Report Date: February 7, 2024Coverage: 1100/1460h (75%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 62 days from Dec 07 to Feb 06

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	60 bpm	49 bpm	81 bpm
Breathing	15 brpm	6 brpm	56 brpm

Limited data in the final 24h (16/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Feb 06	9 AM to 10 AM	1h	1	Elevated Breathing	26	16	56	Check SpO2 and lung sounds; assess for fluid overload or early infection

81 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
81	88h	High Breathing	1	75%

Major Findings: Sustained high breathing (avg 34, peak 60)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jan 26	3 AM to 4 AM	12h	12	Very High Breathing	50	8	60	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 07	9 PM to 10 PM	11h	10	Very High Breathing	49	8	60	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 28	4 PM to 5 PM	4h	4	Very High Breathing	48	14	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Jan 28	6 AM to 7 AM	10h	9	High Breathing	33	7	57	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 09	6 PM to 8 PM	17h	14	High Breathing	34	3	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 27	2 PM to 3 PM	9h	9	High Breathing	35	9	51	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

+ 1 additional condition(s) across Elevated Breathing; details in monitoring data.

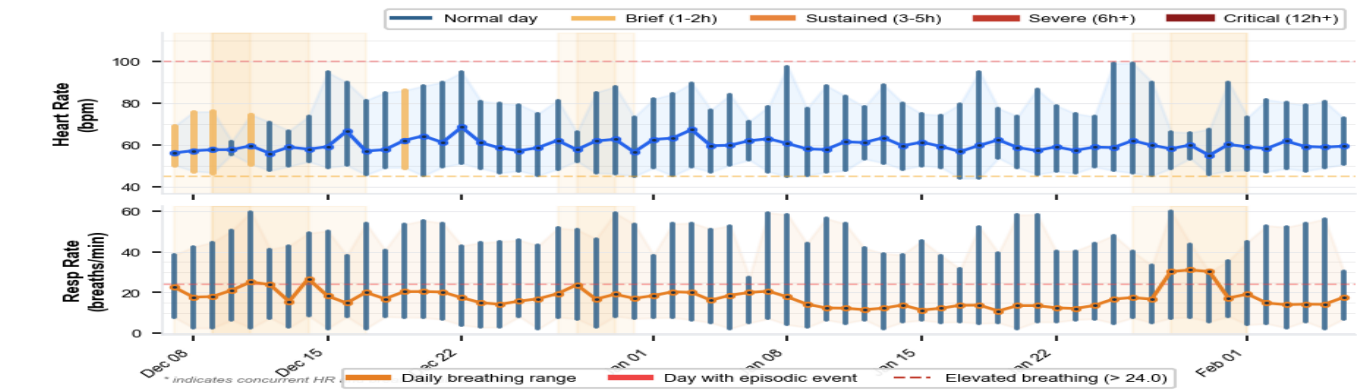
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- High Breathing (avg 34 brpm, peak 50 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 50 brpm, peak 60 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 27 brpm, peak 30 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 7 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

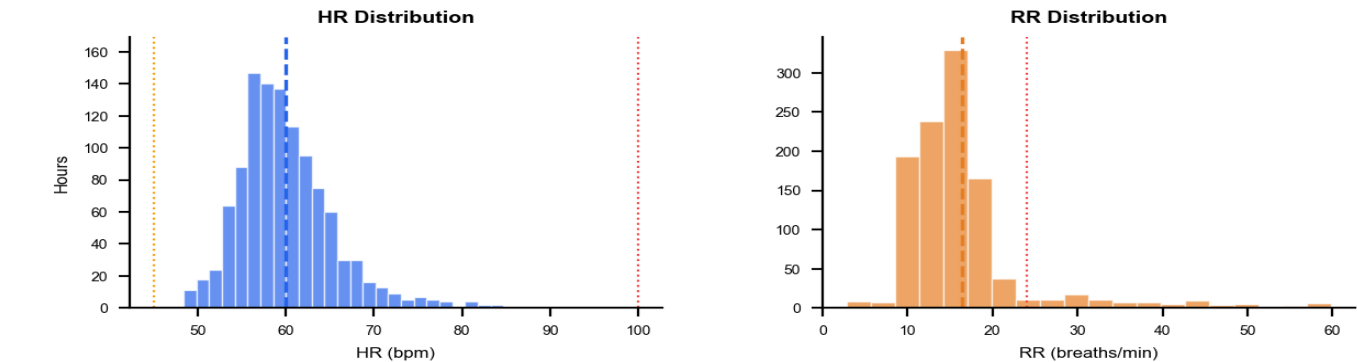
Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 21 brpm (10 to 32).
- 2. **Clustered pattern:** Episodic events on 5 of 62 days (8%).
- 3. **Improving trajectory:** 34 → 23 events vs prior period.

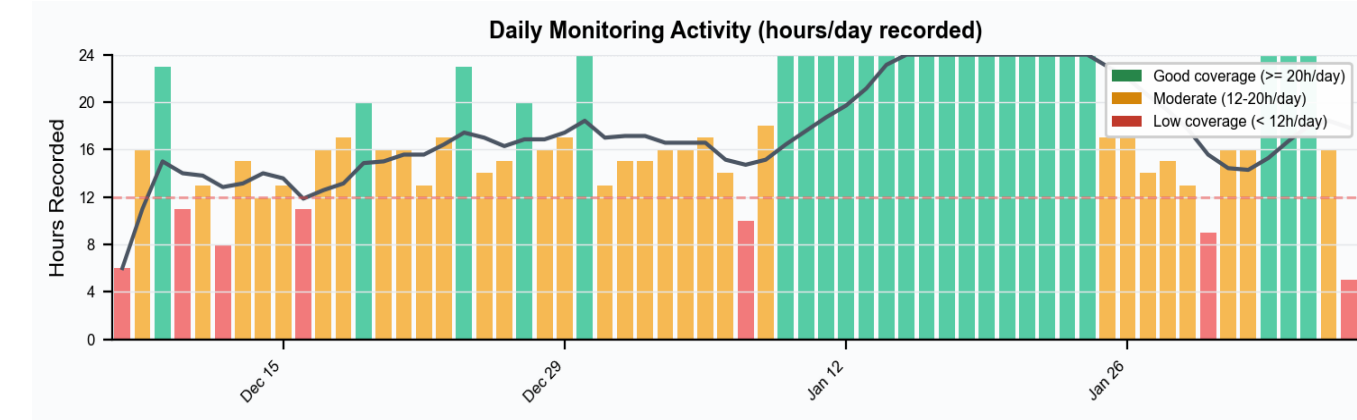


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.